

ENGLISH FOR WORK / SEPTEMBER 2026

Healthcare Administration English



Conversation lab

8 complete workplace dialogues, followed by eight additional role-play cases. Study the exchanges, then make the conversation your own.

PRACTICE THAT TRANSFERS TO WORK

Read a realistic case. Study complete conversations. Find the right language, speak, get feedback, and try again.

Eight lessons | Intermediate to advanced | Independent or classroom study

For hospital administrators, clinic managers, practice administrators, care coordinators, patient-experience leaders, revenue-cycle staff, operations managers, and healthcare-adjacent professionals.

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

All scenarios, figures, and model responses are fictional teaching material. Use invented details in practice.

[Open this course on English Ladder](#)

Inside this conversation lab

Original fictional training conversations in professional English. These are realistic scripts, not transcripts of actual people. The professional roles are the speaker labels; all figures, incidents, and organizations are invented.

01 Referral queue

02 A denied claim

03 Bed capacity huddle

04 Accreditation evidence

05 A vendor data request

06 Service recovery

07 Care coordination handoff

08 Board dashboard definitions

Then try eight independent role-play cases, followed by feedback criteria and references.

Read it. Hear it. Make it yours.

1. Read the setting. Identify the roles, the immediate problem, and what each speaker needs.
2. Read the whole conversation aloud with a partner. In a group, give a third person the observer role. For three-speaker scripts, assign each professional a separate voice.
3. Notice how the speakers ask a specific question, qualify a claim, disagree, explain a technical point, or agree on a next step. Underline language you would actually use.
4. Cover the script. Recreate the exchange using the situation and professional roles. Keep the meaning, but change the wording.
5. Switch roles and introduce a complication: a missing record, a different priority, an unavailable colleague, or a changed assumption. End with a clear outcome or unresolved question.

Register and terminology

These colleagues use concise professional language. In an internal technical meeting, familiar abbreviations can be natural. With a new colleague or a client, expand unfamiliar terms and check understanding. Keep disagreement directed at the evidence or proposal, and match the degree of certainty to what is known.

Independent study

Speak each role aloud. Pause before the reply and supply your own response before revealing the next line. Record a second version using invented details, then compare its clarity and tone with the script.

Professional context

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

FULL DIALOGUE 01 / 6 SPEAKING TURNS

Referral queue

A clinic reviews referrals awaiting appointments.

Roles: Access coordinator / Clinic manager

Access coordinator: We have 18 referrals waiting for triage. Four arrived without the supporting records.

Clinic manager: Separate incomplete referrals from those awaiting a clinical decision. They need different follow-up.

Access coordinator: Can scheduling decide which patients need the earliest slots?

Clinic manager: Clinical urgency belongs with the authorized clinical team. We can identify missing information and available capacity.

Access coordinator: I'll contact the referring offices through the approved channel and update each referral's status.

Clinic manager: I'll ask the triage lead to review the pending queue and confirm the escalation route.

Notice the professional language

Discuss this wording: "Can scheduling decide which patients need the earliest slots?" What does it help the other professional clarify?

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: name one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 02 / 6 SPEAKING TURNS

A denied claim

Revenue-cycle staff investigate a claim denial.

Roles: Billing specialist / Coding supervisor

Billing specialist: This claim was denied for missing authorization. Should I change the diagnosis code and resubmit?

Coding supervisor: No. Coding must reflect the documented encounter. First check the denial reason and authorization record.

Billing specialist: The authorization exists, but its reference wasn't included in the submission.

Coding supervisor: Verify that it applies to the service and dates before correcting the claim.

Billing specialist: I'll retain the denial notice and follow the payer's correction process.

Coding supervisor: Record the submission history so we can distinguish a corrected claim from a duplicate.

Notice the professional language

Discuss this wording: "This claim was denied for missing authorization. Should I change the diagnosis code and resubmit?" What does it help the other professional clarify?

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: name one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 03 / 6 SPEAKING TURNS

Bed capacity huddle

An admissions surge is affecting patient flow.

Roles: Bed manager / Nurse staffing coordinator

Bed manager: The dashboard shows six empty beds. Can we accept six more admissions?

Nurse staffing coordinator: Only three are currently staffed and ready. Two need cleaning, and one has a maintenance restriction.

Bed manager: I'll report physical beds separately from staffed available beds.

Nurse staffing coordinator: Good. I'll confirm the staffing position with the charge nurses before we discuss additional capacity.

Bed manager: Environmental services will give me the turnover status. I'll update the flow huddle after that.

Nurse staffing coordinator: Flag clinical priorities to the care team; the dashboard alone shouldn't determine placement.

Notice the professional language

Discuss this wording: "The dashboard shows six empty beds. Can we accept six more admissions?" What does it help the other professional clarify?

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: name one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 04 / 6 SPEAKING TURNS

Accreditation evidence

A quality team prepares evidence for an accreditation review.

Roles: Quality coordinator / Department manager

Quality coordinator: The checklist says annual training is complete, but three attendance records are missing.

Department manager: The staff say they attended. Can we count that as evidence?

Quality coordinator: We need to verify completion through the approved records, not backdate signatures.

Department manager: I'll check the learning system and ask the training coordinator about alternative documentation.

Quality coordinator: I'll mark the evidence gap clearly and record who owns the follow-up.

Department manager: If completion can't be verified, we'll arrange the required training and document the actual dates.

Notice the professional language

Discuss this wording: "The staff say they attended. Can we count that as evidence?" What does it help the other professional clarify?

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: name one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 05 / 6 SPEAKING TURNS

A vendor data request

An operations vendor requests a patient-level spreadsheet.

Roles: Operations manager / Privacy officer

Operations manager: The vendor wants names and diagnoses to investigate scheduling delays. May I send the full export?

Privacy officer: Let's establish the purpose, permitted access, and minimum information needed before sharing anything.

Operations manager: Could a de-identified summary answer the operational question?

Privacy officer: Possibly. We should assess that and the vendor's current agreements through our privacy process.

Operations manager: I'll ask them to specify the fields and intended use, without attaching patient data.

Privacy officer: Send me that request, and we'll confirm the approved method and scope of any disclosure.

Notice the professional language

Discuss this wording: "The vendor wants names and diagnoses to investigate scheduling delays. May I send the full export?" What does it help the other professional clarify?

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: name one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 06 / 6 SPEAKING TURNS

Service recovery

Patient-experience staff discuss a complaint about repeated cancellations.

Roles: Patient-experience lead / Scheduling supervisor

Patient-experience lead: The patient says we've canceled twice without explaining what happens next.

Scheduling supervisor: The first cancellation was clinician availability. The second was an unresolved referral issue.

Patient-experience lead: We need a clear explanation that doesn't blame the patient or disclose unnecessary information.

Scheduling supervisor: I'll verify the referral status and identify who can resolve the scheduling barrier.

Patient-experience lead: I'll acknowledge the disruption and offer a definite time for an update.

Scheduling supervisor: Once the barrier is resolved, my team will confirm available appointments directly with the patient.

Notice the professional language

Discuss this wording: "The patient says we've canceled twice without explaining what happens next." What does it help the other professional clarify?

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: name one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 07 / 6 SPEAKING TURNS

Care coordination handoff

Two teams clarify responsibility for a discharge follow-up.

Roles: Care coordinator / Clinic administrator

Care coordinator: The discharge note requests follow-up, but I don't see an accepted appointment request.

Clinic administrator: Our referral team received it this morning. Receiving it doesn't mean the appointment is booked.

Care coordinator: Who can confirm the next step and contact the patient?

Clinic administrator: I'll assign the referral coordinator and ask them to acknowledge ownership in the shared record.

Care coordinator: I'll pass on the clinical team's stated timeframe through the approved channel.

Clinic administrator: We'll escalate any inability to meet it to that team and document the response.

Notice the professional language

Discuss this wording: "Who can confirm the next step and contact the patient?" What does it help the other professional clarify?

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: name one natural professional expression and one place where the follow-up made the meaning clearer.

FULL DIALOGUE 08 / 6 SPEAKING TURNS

Board dashboard definitions

Administrators reconcile two reported waiting-time figures.

Roles: Performance analyst / Operations director

Performance analyst: Your slide says a nine-day wait. The access report says fourteen days.

Operations director: Mine measures referral receipt to first offered appointment. What does yours measure?

Performance analyst: Receipt to the appointment the patient actually attends. Different measures answer different questions.

Operations director: Then show both, with the reporting period and exclusions clearly labeled.

Performance analyst: I'll add the denominators and distinguish incomplete referrals from ready-to-schedule cases.

Operations director: Good. The board needs to understand the operational bottleneck before considering additional capacity.

Notice the professional language

Discuss this wording: "Mine measures referral receipt to first offered appointment. What does yours measure?" What does it help the other professional clarify?

Rehearse and extend with AI

Explain the issue and the agreed next step without reading. Identify any point still awaiting evidence, agreement, or approval. Then replay the exchange with a changed constraint and a new ending.

Observer: name one natural professional expression and one place where the follow-up made the meaning clearer.

Eight more situations to make your own

The following cases are open role plays. They give you facts, contrasting roles, useful expressions, and a short model response. Use what you learned from the complete dialogues to create a longer exchange.

Preparation: two minutes. Conversation: three to five minutes. Feedback: one minute. Switch roles and repeat with the second-round challenge.

ROLE-PLAY CASE 01

Patient Access, Scheduling, and Referrals

SHARED CASE

A referral is received without the requested specialty or reason for the appointment. The patient is expecting a booking confirmation.

Role A | Responding professional

Clarify missing information before responding. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You need to know exactly what information is missing. Give one answer only after your partner asks a focused question; then ask them to confirm their understanding.

Useful expressions

When you say ..., do you mean ...?

Could you clarify which ...?

So, my understanding is Is that right?

Cover this until after your first attempt

Could you confirm the specialty and referral details through the approved channel? I can explain the scheduling process to the patient, but I cannot confirm an appointment until the required information is available.

Second round

The other person uses an ambiguous word such as 'ready', 'approved', or 'urgent'. Clarify it before continuing.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 02

Revenue Cycle, Coding, and Denials

SHARED CASE

A dashboard groups rejected submissions and denied claims together. A manager wants to understand where follow-up work is needed.

Role A | Responding professional

Explain a useful distinction in plain English. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You are unfamiliar with one technical term in the case. Ask for a plain-English explanation and then paraphrase it. Do not pretend to understand a term you cannot explain.

Useful expressions

In this context, ... means

The difference is that

How would you explain that distinction to a colleague?

Cover this until after your first attempt

These categories describe different points in the process. Let's separate submissions that did not enter processing from claims that received a denial, then identify the follow-up required for each group.

Second round

Your listener is new to this field. Replace two specialist expressions with clear explanations without changing the meaning.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 03

Patient Flow, Capacity, and Staffing

SHARED CASE

A unit reports 24 occupied beds and two planned discharges. The discharge timing is unconfirmed, but another team assumes two beds are available now.

Role A | Responding professional

Distinguish completed work from pending work and explain its impact. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You need to brief someone who has only one minute. Ask what is complete, what is open, and which open item affects the next action.

Useful expressions

We have completed ...; ... remains open.

As of ..., the status is

The next update will cover

Cover this until after your first attempt

There are 24 occupied beds. Two discharges are planned, but their timing is not confirmed. I'll distinguish current availability from expected availability in the capacity update.

Second round

The listener asks you to reduce the update to two sentences. Preserve the most important completed and pending items.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 04

Quality, Safety, and Accreditation

SHARED CASE

A near-miss report identifies a scheduling error caught before the appointment. The review has not established how the error occurred.

Role A | Responding professional

Separate observations, assumptions, and conclusions. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You need to tell a colleague what is known. Ask which part is confirmed and which part is an assumption. Challenge one statement that sounds more certain than the case supports.

Useful expressions

The available evidence shows

This may indicate ..., but

We have not yet established whether

Cover this until after your first attempt

The error was identified before the appointment. The cause is still under review. I'll record the sequence of events and the checks that caught it without assigning blame or an unconfirmed cause.

Second round

Someone asks for a yes-or-no answer when the evidence supports only a qualified answer. Be concise while preserving the uncertainty.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 05

HIPAA, Privacy, and Information Governance

SHARED CASE

A colleague requests a full patient list for a limited administrative task. The appropriate scope and access have not been confirmed.

Role A | Responding professional

Make a request with a clear action, purpose, and realistic timing. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You can help but need a clear request and its purpose. Ask what is required and whether the timing is fixed or negotiable.

Useful expressions

Could you provide ... so that ...?

Please confirm ... by [agreed time].

If that timing is not possible, please let me know

Cover this until after your first attempt

Could you specify the purpose and fields required? Let's confirm the permitted access and necessary information through our privacy process before sharing the list.

Second round

The other person cannot meet the requested timing. Clarify an alternative without inventing authority to change a formal deadline.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 06

Patient Experience and Service Recovery

SHARED CASE

A patient has called twice about a delayed response. The administrator can investigate the communication delay but cannot make a clinical decision.

Role A | Responding professional

Acknowledge a communication problem and give a corrected message. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You are frustrated because an earlier message created an expectation. Explain that expectation. Ask your partner to distinguish the correction from anything still uncertain.

Useful expressions

My earlier message did not make ... clear.

I'm sorry for The correct information is

Let me check that the revised explanation addresses

Cover this until after your first attempt

I'm sorry you have had to call again. I can check the status of your request and confirm the next contact. For the clinical question, I'll connect you with the appropriate care team.

Second round

The listener remains disappointed after the correction. Acknowledge the impact and restate the available next step calmly.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 07

Population Health and Care Coordination

SHARED CASE

A discharge coordination note lists transport as pending. The next coordinator needs to know who is following up and what has been confirmed.

Role A | Responding professional

Give a handoff that the receiving person can confirm. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You are taking over the work. Ask what remains open and repeat back the critical status or responsibility. Point out one detail that would be unsafe or misleading to assume.

Useful expressions

The current status is ...; the outstanding item is

The record shows

Please confirm who has accepted

Cover this until after your first attempt

Transport remains unconfirmed. The request is recorded, and the assigned coordinator is following up. Please check the latest status with that person and record any change before telling the patient a departure time.

Second round

The intended recipient cannot take ownership. Keep the status clear and identify the need for an authorized reassignment.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

ROLE-PLAY CASE 08

Executive Dashboards and Board Updates

SHARED CASE

A board chart shows waiting time falling from 40 to 30 minutes, but the reporting period changed from monthly to weekly.

Role A | Responding professional

Explain a useful distinction in plain English. Use only the facts given. Choose two relevant terms and be ready to explain one of them in ordinary words.

Role B | Listener

You are unfamiliar with one technical term in the case. Ask for a plain-English explanation and then paraphrase it. Do not pretend to understand a term you cannot explain.

Useful expressions

In this context, ... means

The difference is that

How would you explain that distinction to a colleague?

Cover this until after your first attempt

The reported waiting time is lower, but the periods differ. I'll label the time windows and prepare a comparable view before we describe the change as a sustained improvement.

Second round

Your listener is new to this field. Replace two specialist expressions with clear explanations without changing the meaning.

Debrief and extend with AI

Which phrase helped the listener? Which case fact needed clarification? Say one better follow-up question, then repeat the exchange.

Lesson conversations and speaking practice

The following sections match 05 and 06 on the industry webpage and in the learner workbook. Each lesson has three ten-turn conversations and two bounded speaking scenarios. These are original fictional teaching scripts, not transcripts of real people.

LESSON 01 / 05 • CONVERSATIONS / 1 OF 3

A referral without a specialty

Fictional scheduling call: a referral lacks the requested specialty and appointment reason, while the patient expects confirmation.

01 • Access coordinator: This referral has no specialty or appointment reason. Could you clarify what service was requested?

02 • Referral clerk: I can check with the referring clinician, but I can't determine the specialty myself.

03 • Access coordinator: The patient expects a booking confirmation. Should I select a department from their previous visits?

04 • Referral clerk: No. That would substitute an assumption for the missing referral information.

05 • Access coordinator: I'll explain that we received the referral and are seeking the details needed for scheduling.

06 • Referral clerk: Please keep that separate from eligibility or authorization status, which we haven't checked here.

07 • Access coordinator: Can you ask for the specialty and reason, then return the confirmed information to me?

08 • Referral clerk: Yes. I'll contact the referring team and identify your request as a scheduling clarification.

09 • Access coordinator: I'll remain the contact for the booking update without promising an appointment time.

10 • Referral clerk: Understood. I'll send the clarification through the designated referral channel once the team responds.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 01 / 05 • CONVERSATIONS / 2 OF 3

Eligibility checked, authorization still unknown

Separate fictional access case: an eligibility check is complete for a scheduled service, but authorization status is blank.

01 • Scheduling supervisor: The eligibility check is complete. Can we tell the department authorization is confirmed too?

02 • Authorization specialist: No. Those fields describe different checks, and the authorization status is currently blank.

03 • Scheduling supervisor: Does blank mean authorization is unnecessary for this appointment, or that it was denied?

04 • Authorization specialist: Neither conclusion is supported. We need the applicable requirements and the case status verified.

05 • Scheduling supervisor: Which details do you need to perform that check without requesting the entire patient record?

06 • Authorization specialist: Please provide the service reference and scheduled date through our authorized work queue.

07 • Scheduling supervisor: Can you confirm which status wording I should use while your check is pending?

08 • Authorization specialist: Say eligibility checked, authorization status not yet confirmed. Avoid any promise about payment.

09 • Scheduling supervisor: I'll use those separate labels and direct follow-up questions to your queue.

10 • Authorization specialist: I'll return the verified status and identify any additional information needed before the scheduling team responds.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 01 / 05 • CONVERSATIONS / 3 OF 3

A cancelled visit counted as a no-show

Separate fictional appointment review: a cancellation message arrived before the visit, but the appointment is marked no-show; receipt time is recorded.

01 • Patient access analyst: This visit is marked no-show, but a cancellation message arrived before the appointment.

02 • Scheduling lead: Have you checked whether the message refers to this visit rather than another booking?

03 • Patient access analyst: The appointment reference matches. I haven't checked how the status was entered.

04 • Scheduling lead: Then let's clarify the workflow before assuming someone ignored the cancellation message.

05 • Patient access analyst: Could this affect our reported no-show count and the patient's future contact?

06 • Scheduling lead: It could. We should verify the appropriate classification under our scheduling process.

07 • Patient access analyst: Will you review the message timestamp and appointment history with the responsible team?

08 • Scheduling lead: Yes. Please keep the original record available so any correction has a clear basis.

09 • Patient access analyst: I'll flag this entry for review rather than silently changing the dashboard total.

10 • Scheduling lead: I'll confirm the outcome and make sure the access team receives the corrected status if needed.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 01 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Patient Access, Scheduling, and Referrals

A referral is received without the requested specialty or reason for the appointment. The patient is expecting a booking confirmation.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You need to know exactly what information is missing. Give one answer only after your partner asks a focused question; then ask them to confirm their understanding.

Second round: The other person uses an ambiguous word such as 'ready', 'approved', or 'urgent'. Clarify it before continuing.

Model for scenario 1: Could you confirm the specialty and referral details through the approved channel? I can explain the scheduling process to the patient, but I cannot confirm an appointment until the required information is available.

Scenario 2 | Two referral entries, one uncertain request

A fictional scheduling queue contains two referrals with the same patient reference and date but different originating departments. Neither entry says whether it replaces the other. Discuss what must be clarified before booking.

Role A: Referral coordinator: ask whether these are duplicate or separate requests.

Role B: Referring-office administrator: verify the intended requests with the clinical team without choosing a specialty yourself.

Possible opening: These entries may be duplicates, but the different departments mean I need confirmation before booking.

Success checks

- Do not infer duplication from matching dates alone.
- Ask which request remains active and what service is intended.
- Agree who returns the verified scheduling instruction.

Add a complication: A colleague proposes deleting the later entry to shorten the queue.

LESSON 02 / 05 • CONVERSATIONS / 1 OF 3

Rejected submissions and denied claims need different queues

Fictional revenue-cycle review: the dashboard combines submissions rejected before processing and claims denied after review.

01 • Operations manager: Why can't we assign every item in this dashboard to the denial team?

02 • Revenue cycle analyst: It combines rejected submissions with denied claims, although their follow-up paths may differ.

03 • Operations manager: Could you explain that distinction without assuming I know the processing terminology?

04 • Revenue cycle analyst: In this dataset, rejected items didn't enter processing; denied claims received an adverse payment decision.

05 • Operations manager: So correcting a submission field isn't necessarily the same task as reviewing a denial reason?

06 • Revenue cycle analyst: Exactly. We need the actual response codes and local workflow before assigning each item.

07 • Operations manager: Does a corrected submission automatically become a paid claim once it is accepted?

08 • Revenue cycle analyst: No. Acceptance for processing doesn't guarantee payment or settle coding questions.

09 • Operations manager: I'll ask for separate totals and owners rather than one undifferentiated problem count.

10 • Revenue cycle analyst: I'll split the queue by verified status and retain items whose response codes still need interpretation.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 02 / 05 • CONVERSATIONS / 2 OF 3

A clean claim is not a payment guarantee

Separate fictional billing meeting: twenty submissions pass the team's completeness checks, but payer outcomes are not available.

01 • Billing supervisor: Twenty claims passed our checks. May I describe them as twenty confirmed payments?

02 • Claims specialist: No. They passed completeness checks; we haven't received the payer outcomes yet.

03 • Billing supervisor: Then what does clean claim mean in this particular internal report?

04 • Claims specialist: It means the submissions met our specified checks for processing, not that payment was promised.

05 • Billing supervisor: Could there still be a coding or coverage question after those checks?

06 • Claims specialist: Yes, depending on the case. We should use actual responses rather than predict a denial or payment.

07 • Billing supervisor: How should I explain the progress to the department without overstating collections?

08 • Claims specialist: Say twenty checked submissions are ready for the next processing stage and outcomes remain pending.

09 • Billing supervisor: I'll separate submission quality from payment status in the meeting summary.

10 • Claims specialist: I'll confirm the response status later and keep the report's clean-claim definition attached to the metric.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 02 / 05 • CONVERSATIONS / 3 OF 3

Denial count and denial value tell different stories

Separate fictional dashboard exercise: ten denied claims total 1,000 units of currency; two other denied claims total 8,000, with no recovery estimates supplied.

01 • Finance partner: The first group has more denials. Should it automatically receive all our follow-up capacity?

02 • Denials analyst: Not necessarily. Ten claims total one thousand, while the other two total eight thousand.

03 • Finance partner: So the count measures workload, while the amount shows the value involved?

04 • Denials analyst: Those are useful distinctions, although complexity and recoverability also matter and remain unknown here.

05 • Finance partner: Can we assume the larger amount is entirely recoverable if we act quickly?

06 • Denials analyst: No. We need the denial reasons and supporting records before estimating an outcome.

07 • Finance partner: What should the team compare before proposing a work order for these groups?

08 • Denials analyst: Claim count, value, verified deadlines, and case complexity, with unknowns clearly labeled.

09 • Finance partner: I'll avoid ranking the groups by one headline number and ask for that assessment.

10 • Denials analyst: I'll bring a scoped comparison without converting the denied amount into a promised recovery.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 02 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Revenue Cycle, Coding, and Denials

A dashboard groups rejected submissions and denied claims together. A manager wants to understand where follow-up work is needed.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You are unfamiliar with one technical term in the case. Ask for a plain-English explanation and then paraphrase it. Do not pretend to understand a term you cannot explain.

Second round: Your listener is new to this field. Replace two specialist expressions with clear explanations without changing the meaning.

Model for scenario 1: These categories describe different points in the process. Let's separate submissions that did not enter processing from claims that received a denial, then identify the follow-up required for each group.

Scenario 2 | A returned claim and a suspected coding problem

A fictional submission was returned for a missing identifier. A manager calls it a coding denial, but no adjudication response or coding review exists. Explain the known processing issue and route the next action.

Role A: Billing analyst: distinguish the returned submission from an established coding denial.

Role B: Department administrator: ask what needs correcting and what remains unassessed.

Possible opening: The available response identifies a missing identifier; it does not establish a coding denial.

Success checks

- State the observed response without adding a coding conclusion.
- Separate correction of submission data from review of clinical coding.
- Agree the appropriate queue and a verified status label.

Add a complication: The manager wants the item included in a coding-error presentation that afternoon.

LESSON 03 / 05 • CONVERSATIONS / 1 OF 3

Two planned discharges are not two available beds

Fictional capacity call: the unit has 24 occupied beds and two planned discharges whose timing is unconfirmed.

01 • Bed manager: Your unit expects two discharges. Can I show two beds available right now?

02 • Unit administrator: No. Our current census is twenty-four occupied beds, and both discharge times are unconfirmed.

03 • Bed manager: The transfer team interpreted planned as completed. What should I tell them instead?

04 • Unit administrator: Say two discharges are anticipated, but current bed availability has not changed.

05 • Bed manager: Would departure alone establish that a bed is ready for the next patient?

06 • Unit administrator: We also need the relevant readiness checks under our local bed-management process.

07 • Bed manager: I'll keep the pending movements separate from confirmed capacity on the board.

08 • Unit administrator: Thank you. I'll check the status with the responsible clinical and operational teams.

09 • Bed manager: Please update me when a change is confirmed, including any staffing dependency.

10 • Unit administrator: I will. I won't turn an expected discharge into an available-bed promise before readiness is established.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 03 / 05 • CONVERSATIONS / 2 OF 3

Physical space without confirmed staffing

Separate fictional capacity exercise: a room is prepared, but staffing coverage for an additional admission has not been confirmed.

01 • Capacity coordinator: The room is ready. Why does the capacity board still show admission readiness pending?

02 • Staffing coordinator: Because staffing coverage for an additional admission has not yet been confirmed.

03 • Capacity coordinator: Could we count the room as staffed capacity and resolve the roster later?

04 • Staffing coordinator: That would hide an important dependency. Physical space and operational capacity aren't the same status.

05 • Capacity coordinator: Should I quote a standard staffing ratio to settle the question in this exercise?

06 • Staffing coordinator: No universal ratio is supplied here. The authorized team must assess the applicable staffing requirements.

07 • Capacity coordinator: I'll report room preparation complete and staffing confirmation pending, with no admission promise.

08 • Staffing coordinator: I'll ask the responsible supervisor for the coverage assessment and expected update time.

09 • Capacity coordinator: Can you return a confirmed status directly to bed management when you receive it?

10 • Staffing coordinator: Yes. I'll close that loop so the board reflects usable capacity rather than room count alone.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 03 / 05 • CONVERSATIONS / 3 OF 3

A throughput update with an unresolved transport step

Separate fictional flow exercise: discharge paperwork is complete for one planned departure, but transport arrival has not been confirmed.

01 • Flow manager: Is the discharge complete now that the paperwork is finished for this case?

02 • Discharge administrator: Only the paperwork step is complete. Transport arrival is still unconfirmed.

03 • Flow manager: The throughput report needs a status before the afternoon call. What can we safely report?

04 • Discharge administrator: Administrative preparation complete, departure pending transport confirmation. That states the completed work and the blocker.

05 • Flow manager: Can we use the requested collection time as the actual departure time?

06 • Discharge administrator: No. A requested time is not an observed departure or a carrier confirmation.

07 • Flow manager: Who is following up so the receiving team doesn't make a duplicate request?

08 • Discharge administrator: I'll contact the transport coordinator and remain responsible for the administrative status update.

09 • Flow manager: Please distinguish the booking status from any clinical decision about discharge readiness.

10 • Discharge administrator: Understood. I'll report the operational facts and refer clinical readiness questions to the responsible clinician.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 03 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Patient Flow, Capacity, and Staffing

A unit reports 24 occupied beds and two planned discharges. The discharge timing is unconfirmed, but another team assumes two beds are available now.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You need to brief someone who has only one minute. Ask what is complete, what is open, and which open item affects the next action.

Second round: The listener asks you to reduce the update to two sentences. Preserve the most important completed and pending items.

Model for scenario 1: There are 24 occupied beds. Two discharges are planned, but their timing is not confirmed. I'll distinguish current availability from expected availability in the capacity update.

Scenario 2 | A census snapshot taken at a different time

In a fictional morning briefing, the overnight report shows eighteen occupied beds at 06:00 and the live board shows twenty at 08:00. Movement records have not been reconciled. Discuss the update without treating either count as an error.

Role A: Capacity analyst: explain the snapshot times and request reconciliation.

Role B: Unit coordinator: identify the need to check movements before updating the executive report.

Possible opening: These counts were taken two hours apart, so the difference does not by itself establish an error.

Success checks

- State both counts with their times.
- Separate a possible movement difference from a confirmed data error.
- Agree who verifies the current census before it is circulated.

Add a complication: The chair asks you to use the lower number because it looks better.

LESSON 04 / 05 • CONVERSATIONS / 1 OF 3

An intercepted scheduling error with no established cause

Fictional safety review: a scheduling error was caught before the appointment, and the cause remains unknown.

01 • Quality coordinator: The near miss involved a scheduling error caught before the appointment took place.

02 • Service manager: Can the report say a distracted scheduler caused it so we can assign corrective action?

03 • Quality coordinator: We haven't established that. The current report describes the error and its interception.

04 • Service manager: What evidence would help us understand how the incorrect booking entered the system?

05 • Quality coordinator: The audit trail, source request, and relevant staff accounts could help establish the sequence.

06 • Service manager: Should we leave the report open while those sources are being checked?

07 • Quality coordinator: Yes. We can record known facts without presenting a suspected cause as a finding.

08 • Service manager: I'll ask the team to preserve the records and avoid blame in the briefing.

09 • Quality coordinator: I'll distinguish immediate containment from any corrective action that depends on the review.

10 • Service manager: Then we'll discuss the verified sequence before deciding what process change is appropriate.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 04 / 05 • CONVERSATIONS / 2 OF 3

Missing evidence is not proof the check never happened

Separate fictional accreditation preparation: three sampled files lack a documented administrative check; whether the checks occurred is unknown.

01 • Accreditation coordinator: Three sampled files lack documentation of the administrative check. What should our finding say?

02 • Department lead: Could we state that staff skipped all three checks and schedule retraining?

03 • Accreditation coordinator: We know the evidence is missing. We don't yet know whether the activity occurred.

04 • Department lead: But doesn't the missing entry make this a confirmed documentation issue at least?

05 • Accreditation coordinator: Yes. We can describe the missing entries accurately and investigate the underlying process separately.

06 • Department lead: Would checking the audit trail help distinguish a recording problem from an omitted task?

07 • Accreditation coordinator: It may help. We should also ask the responsible team without suggesting the desired answer.

08 • Department lead: I'll keep the sample size visible so nobody assumes the finding covers every file.

09 • Accreditation coordinator: I'll request the supporting records and separate verified gaps from possible explanations.

10 • Department lead: We'll choose the corrective action after reviewing that evidence, not label retraining as an established solution today.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 04 / 05 • CONVERSATIONS / 3 OF 3

Fewer reports do not automatically mean fewer errors

Separate fictional quality meeting: near-miss reports fell from twelve to five after the reporting portal changed; reporting behavior has not been assessed.

01 • Operations director: Reports fell from twelve to five. Can I call the new portal a safety improvement?

02 • Quality analyst: Not yet. We know the report count changed, not whether underlying errors became less frequent.

03 • Operations director: What else could explain the difference besides safer work on the front line?

04 • Quality analyst: Reporting behavior may have changed, but that's a hypothesis we still need to assess.

05 • Operations director: So we should avoid saying the portal caused either better safety or underreporting?

06 • Quality analyst: Exactly. Both would go beyond the available evidence in this case.

07 • Operations director: What follow-up would give the review a clearer basis for interpreting the count?

08 • Quality analyst: Check comparable periods, portal access issues, and staff experience with submitting near misses.

09 • Operations director: I'll present the numbers as a reporting trend and keep the safety conclusion open.

10 • Quality analyst: I'll bring the follow-up evidence and distinguish the number of reports from the number of events.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 04 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Quality, Safety, and Accreditation

A near-miss report identifies a scheduling error caught before the appointment. The review has not established how the error occurred.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You need to tell a colleague what is known. Ask which part is confirmed and which part is an assumption. Challenge one statement that sounds more certain than the case supports.

Second round: Someone asks for a yes-or-no answer when the evidence supports only a qualified answer. Be concise while preserving the uncertainty.

Model for scenario 1: The error was identified before the appointment. The cause is still under review. I'll record the sequence of events and the checks that caught it without assigning blame or an unconfirmed cause.

Scenario 2 | A repeated error with different booking routes

Two fictional scheduling errors occurred this week, one through an online form and one by telephone. The final wrong department was the same. Discuss what can be concluded before a common root cause has been assessed.

Role A: Quality reviewer: state the shared outcome and different routes.

Role B: Scheduling manager: propose evidence gathering without declaring a single cause.

Possible opening: The errors reached the same department, but they entered through different routes.

Success checks

- Separate a common outcome from a demonstrated common cause.
- Request the relevant audit trails and source instructions.
- Keep corrective-action effectiveness unconfirmed until reviewed.

Add a complication: A colleague wants to close both cases under a single untested corrective action.

LESSON 05 / 05 • CONVERSATIONS / 1 OF 3

Request the task scope before the full patient list

Fictional administrative request: a colleague asks for a full patient list for a limited task, and access scope is unconfirmed.

01 • Clinic administrator: Can you send the full patient list so I can complete this administrative task?

02 • Information governance officer: Please clarify the task, intended recipients, and fields you need before we consider the request.

03 • Clinic administrator: Would sharing everything now be faster than checking each field with the team?

04 • Information governance officer: It could disclose information beyond the task. We need the appropriate scope and access confirmed.

05 • Clinic administrator: Are you saying HIPAA always prohibits using a patient list for administration?

06 • Information governance officer: No. I'm asking for a case-specific review, not making a blanket legal statement.

07 • Clinic administrator: I'll describe the purpose and request only the information needed for that purpose.

08 • Information governance officer: Please use the approved access-request channel and identify when the task must be completed.

09 • Clinic administrator: I'll submit that today and wait for a verified sharing route rather than email the list.

10 • Information governance officer: I'll route it for review and clarify any minimum-necessary questions under the applicable process.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 05 / 05 • CONVERSATIONS / 2 OF 3

A vendor requests a sample file

Separate fictional privacy exercise: an unverified external vendor requests a patient-data sample for a demonstration; no sharing approval exists.

01 • Procurement coordinator: The vendor wants a sample file for tomorrow's demonstration. Could we send a few patient rows?

02 • Privacy officer: Please don't send protected health information before the purpose, access, and sharing conditions are reviewed.

03 • Procurement coordinator: They called themselves a business associate. Does that statement settle the approval question?

04 • Privacy officer: No. A vendor's label isn't verification of the relevant arrangement or permitted data use.

05 • Procurement coordinator: Would a synthetic demonstration file avoid using real patient information while we clarify this?

06 • Privacy officer: That may meet the demonstration need. Confirm the file is genuinely synthetic and suitable for the task.

07 • Procurement coordinator: I'll ask the vendor which functions they need to demonstrate rather than offer live records.

08 • Privacy officer: Please also request their intended data handling and route the proposal through the responsible reviewers.

09 • Procurement coordinator: I'll make that request today and keep the real sample out of the exchange.

10 • Privacy officer: Good. I'll help identify the review owner without treating tomorrow's meeting as permission to disclose.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 05 / 05 • CONVERSATIONS / 3 OF 3

Remove identifiers without claiming anonymity

Separate fictional reporting exercise: a draft administrative extract omits names but retains patient numbers and detailed appointment histories.

01 • Reporting analyst: I removed the names. Can I send this extract to the wider planning group?

02 • Privacy specialist: Please request a sharing review first. Patient numbers and detailed histories still remain in the file.

03 • Reporting analyst: I thought removing names meant the information was no longer identifiable or protected.

04 • Privacy specialist: That conclusion isn't established. Other fields or combinations can still create privacy concerns.

05 • Reporting analyst: What should I provide so the reviewer can assess the request efficiently?

06 • Privacy specialist: Describe the planning purpose, recipients, required fields, and proposed delivery method without circulating the extract broadly.

07 • Reporting analyst: Could a grouped count meet the purpose instead of sharing individual appointment histories?

08 • Privacy specialist: Ask the planning owner to confirm that. A smaller output may be sufficient, but it still needs assessment.

09 • Reporting analyst: I'll request that confirmation and keep the source extract in its restricted location.

10 • Privacy specialist: I'll coordinate the review and distinguish reduced detail from any verified claim that the output is anonymous.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 05 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | HIPAA, Privacy, and Information Governance

A colleague requests a full patient list for a limited administrative task. The appropriate scope and access have not been confirmed.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You can help but need a clear request and its purpose. Ask what is required and whether the timing is fixed or negotiable.

Second round: The other person cannot meet the requested timing. Clarify an alternative without inventing authority to change a formal deadline.

Model for scenario 1: Could you specify the purpose and fields required? Let's confirm the permitted access and necessary information through our privacy process before sharing the list.

Scenario 2 | An old folder permission follows a new role

A fictional administrator has moved to another department but can still open a patient-report folder used in their previous role. No new access need has been established. Hold a call requesting an access review without opening additional records.

Role A: Department administrator: report the remaining access and request confirmation of the appropriate permission.

Role B: Access-control specialist: ask for the folder reference and new role through the approved channel.

Possible opening: My previous patient-report access still works, but I need you to review whether it fits my new role.

Success checks

- State the access issue without browsing unnecessary patient records.
- Request a role-specific permission review.
- Agree a responsible reviewer and avoid claiming a legal conclusion.

Add a complication: A coworker suggests retaining access in case it becomes useful later.

LESSON 06 / 05 • CONVERSATIONS / 1 OF 3

Repairing a promised callback message

Fictional service-recovery review: a patient has called twice about a delayed response, and the administrator cannot make clinical decisions.

01 • Patient experience lead: The patient has called twice. What explanation did our team provide about the delayed response?

02 • Clinic administrator: We said someone would respond, but we didn't give a clear owner for follow-up.

03 • Patient experience lead: How can we acknowledge that communication problem without promising a clinical decision ourselves?

04 • Clinic administrator: I'll apologize for the unclear follow-up and explain that I can investigate the communication delay.

05 • Patient experience lead: The patient may ask whether the delayed response means their request was refused.

06 • Clinic administrator: I'll clarify that I don't have a confirmed clinical decision and won't infer one from the delay.

07 • Patient experience lead: What concrete service-recovery step can you personally take and explain accurately?

08 • Clinic administrator: I'll contact the responsible team and confirm who will provide the next update.

09 • Patient experience lead: Please distinguish that update from a promise that the underlying matter will be resolved.

10 • Clinic administrator: Agreed. I'll own the communication follow-up and use the appropriate grievance route if requested.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 06 / 05 • CONVERSATIONS / 2 OF 3

Correcting a guaranteed appointment message

Separate fictional patient-experience case: an administrator said an appointment was guaranteed, but the record shows only a requested slot.

01 • Scheduling manager: Your earlier message said guaranteed, but the record shows the appointment is only requested.

02 • Access administrator: I need to correct that clearly and acknowledge that my wording created the wrong expectation.

03 • Scheduling manager: What will you say without blaming the patient for misunderstanding the message?

04 • Access administrator: I said guaranteed when I should have said requested. The appointment is not yet confirmed.

05 • Scheduling manager: Will you promise another slot immediately to repair the patient experience?

06 • Access administrator: Not unless availability is verified. I'll check the request and explain the confirmed options.

07 • Scheduling manager: Please also acknowledge the practical inconvenience caused by the earlier message.

08 • Access administrator: I will, and I'll take responsibility for returning an accurate scheduling update.

09 • Scheduling manager: How will you make sure the next colleague doesn't repeat the original promise?

10 • Access administrator: I'll correct the status through our process and hand over the communication issue without erasing the original history.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 06 / 05 • CONVERSATIONS / 3 OF 3

A survey score is not this person's resolved concern

Separate fictional service review: a department cites an improved HCAHPS score while one patient's reported communication concern remains unresolved.

01 • Service director: Our HCAHPS score improved. Can we tell the patient that communication problems have been fixed?

02 • Patient experience specialist: No. The survey trend doesn't resolve this person's outstanding concern or explain what happened.

03 • Service director: How should I repair that message if I already mentioned the score as reassurance?

04 • Patient experience specialist: Acknowledge that the general result didn't address their experience, then return to their specific concern.

05 • Service director: Could I say their experience must have been unusual because the score improved?

06 • Patient experience specialist: That would dismiss the concern without investigating it. We don't know how representative it is.

07 • Service director: I'll say we still need to review the missed communication and confirm a follow-up owner.

08 • Patient experience specialist: Good. Explain the service-recovery or grievance route available through our organization without promising a finding.

09 • Service director: I'll separate the department metric from the individual's case in the conversation.

10 • Patient experience specialist: And I'll coordinate the case follow-up so the apology is accompanied by a clear next action.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 06 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Patient Experience and Service Recovery

A patient has called twice about a delayed response. The administrator can investigate the communication delay but cannot make a clinical decision.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You are frustrated because an earlier message created an expectation. Explain that expectation. Ask your partner to distinguish the correction from anything still uncertain.

Second round: The listener remains disappointed after the correction. Acknowledge the impact and restate the available next step calmly.

Model for scenario 1: I'm sorry you have had to call again. I can check the status of your request and confirm the next contact. For the clinical question, I'll connect you with the appropriate care team.

Scenario 2 | Directions to the wrong entrance

A fictional appointment reminder directed visitors to Entrance A, which is closed for this training case; the confirmed open entrance is B. A patient missed check-in while looking for A. Discuss a corrective phone response without promising clinical rescheduling.

Role A: Patient access supervisor: acknowledge the incorrect directions and provide the verified entrance information.

Role B: Patient liaison: ask how the appointment question will reach the appropriate scheduling team.

Possible opening: Our reminder gave the wrong entrance; Entrance B is the confirmed route, and I'm sorry for the inconvenience.

Success checks

- Accept the communication error without blaming the patient.
- Give the verified correction and distinguish it from rescheduling authority.
- Agree who reviews the appointment status and contacts the patient.

Add a complication: A colleague wants to describe the patient as a no-show without reviewing the circumstances.

LESSON 07 / 05 • CONVERSATIONS / 1 OF 3

Transport pending at the coordination handoff

Fictional discharge-coordination handoff: the note lists transport as pending, with no confirmed collection time.

01 • Outgoing coordinator: Transport remains pending in this discharge-planning case; I don't have a confirmed collection time.

02 • Incoming coordinator: Who has been following up, and what information can I rely on now?

03 • Outgoing coordinator: The only confirmed status here is pending. I'll pass the request reference so you can verify ownership.

04 • Incoming coordinator: Please don't describe transport as booked unless the provider has actually confirmed it.

05 • Outgoing coordinator: Agreed. I'll distinguish the request from confirmation and avoid implying a clinical discharge decision.

06 • Incoming coordinator: I'll take ownership of the next administrative follow-up once I receive the reference.

07 • Outgoing coordinator: Can you read back the unresolved item so we know the handoff is clear?

08 • Incoming coordinator: Collection timing and booking confirmation are unresolved; I will check them with the responsible service.

09 • Outgoing coordinator: Thank you. Please update the care-coordination record when you receive a verified response.

10 • Incoming coordinator: I will, and I'll flag any remaining barrier rather than inventing a transport arrangement.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 07 / 05 • CONVERSATIONS / 2 OF 3

A care-gap list is not a confirmed missed service

Separate fictional population-health handoff: a dashboard flags six possible care gaps, but outside-provider records have not been reconciled.

01 • Population health analyst: I'm handing over six possible care gaps from the dashboard for reconciliation.

02 • Care coordination lead: Have we established that the services were missed, or only that our records lack them?

03 • Population health analyst: Only the latter. Outside-provider records haven't been reconciled with this list.

04 • Care coordination lead: Then we shouldn't contact people saying they definitely failed to receive care.

05 • Population health analyst: Correct. I'll label the flags as unverified and provide the source dates.

06 • Care coordination lead: Who will check the authorized records before deciding what outreach is appropriate?

07 • Population health analyst: Could your coordination team own that review and return any confirmed corrections?

08 • Care coordination lead: Yes. I'll assign the review and keep clinical interpretation with the responsible clinicians.

09 • Population health analyst: Please also distinguish resolved data gaps from any actual care gaps that remain.

10 • Care coordination lead: Understood. I'll report both separately and won't treat a missing entry as proof of a missed service.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 07 / 05 • CONVERSATIONS / 3 OF 3

Handing over an unresolved access barrier

Separate fictional coordination exercise: a person reports difficulty reaching follow-up appointments; the specific barrier and available assistance are unconfirmed.

01 • Community liaison: The person reported difficulty reaching follow-up appointments. I'm handing over that concern for clarification.

02 • Care coordinator: Do we know whether the issue is transport, timing, cost, or something else?

03 • Community liaison: No. I haven't established the specific barrier, and I don't want to label it incorrectly.

04 • Care coordinator: Good. Social determinants are relevant context, but we shouldn't substitute assumptions for the person's account.

05 • Community liaison: Can you own the follow-up conversation and ask what assistance would actually be useful?

06 • Care coordinator: Yes. I'll clarify the barrier and check available services without promising support we haven't confirmed.

07 • Community liaison: Should the handoff link this issue to a predicted readmission for that person?

08 • Care coordinator: No. We have no basis for that individual prediction in the supplied information.

09 • Community liaison: I'll preserve their wording and identify the unresolved access question in the coordination note.

10 • Care coordinator: I'll confirm receipt and report the agreed next step through the appropriate care-coordination channel.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 07 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Population Health and Care Coordination

A discharge coordination note lists transport as pending. The next coordinator needs to know who is following up and what has been confirmed.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You are taking over the work. Ask what remains open and repeat back the critical status or responsibility. Point out one detail that would be unsafe or misleading to assume.

Second round: The intended recipient cannot take ownership. Keep the status clear and identify the need for an authorized reassignment.

Model for scenario 1: Transport remains unconfirmed. The request is recorded, and the assigned coordinator is following up. Please check the latest status with that person and record any change before telling the patient a departure time.

Scenario 2 | A follow-up referral sent but not received

In a fictional discharge-coordination case, an outgoing referral was sent yesterday. The receiving service has not acknowledged it, and no appointment is confirmed. Practice transferring follow-up responsibility at shift change.

Role A: Outgoing coordinator: distinguish transmission from acceptance and hand over the reference.

Role B: Incoming coordinator: confirm ownership of checking receipt and appointment status.

Possible opening: The referral was sent yesterday, but receipt and appointment arrangements are still unconfirmed.

Success checks

- Separate sent, received, and scheduled statuses.
- Identify the receiving owner and unresolved questions.
- Avoid promising an appointment while agreeing a clear follow-up action.

Add a complication: The family asks you to say that follow-up is fully arranged.

LESSON 08 / 05 • CONVERSATIONS / 1 OF 3

A monthly wait compared with a weekly wait

Fictional board preparation: a waiting-time chart falls from 40 to 30 minutes, but the reporting interval changed from monthly to weekly.

01 • Board secretary: The chart drops from forty to thirty minutes. Can the headline say waiting time improved?

02 • Performance analyst: We need a qualification: the earlier figure is monthly, while the newer figure is weekly.

03 • Board secretary: Why does that matter when both figures use minutes and the same KPI name?

04 • Performance analyst: They summarize different time windows, which may contain different activity and variation.

05 • Board secretary: So the visible difference is not automatically a like-for-like performance improvement?

06 • Performance analyst: Correct. We should align the periods or clearly explain why the comparison is limited.

07 • Board secretary: Would labeling the reporting intervals beside the figures be enough for today's discussion?

08 • Performance analyst: It would help, but don't retain a causal or sustained-improvement claim unsupported by comparable data.

09 • Board secretary: I'll revise the headline and ask the metric owner for aligned periods.

10 • Performance analyst: I'll provide a comparable series and keep the current figures identified as different reporting windows.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 08 / 05 • CONVERSATIONS / 2 OF 3

Target variance versus change from last month

Separate fictional dashboard: response time is eight days, the target is six, and last month's result was ten.

01 • Executive assistant: The result is eight days, down from ten. Why is the dashboard still showing an unfavorable variance?

02 • KPI owner: Because the target is six days. Improvement from last month and variance from target are different comparisons.

03 • Executive assistant: So we improved by two days but remain two days above the target?

04 • KPI owner: Exactly. Both statements can be true without changing the underlying result.

05 • Executive assistant: Could I describe the target as achieved because the direction is favorable?

06 • KPI owner: No. Direction of change doesn't erase the remaining gap to the stated target.

07 • Executive assistant: What should the board hear about accountability rather than just the color on the dashboard?

08 • KPI owner: Who owns the measure, what the gap is, and which follow-up remains under review.

09 • Executive assistant: I'll explain both comparisons and avoid claiming a cause for the improvement.

10 • KPI owner: I'll bring the supporting period definitions and a status update on the review, without inventing a corrective-action outcome.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 08 / 05 • CONVERSATIONS / 3 OF 3

A department average hides different queues

Separate fictional executive exercise: one queue averages ten minutes and another fifty; the combined figure lacks queue volumes.

01 • Chief operating officer: Can we average ten and fifty and report thirty minutes for the department?

02 • Dashboard analyst: Not without the queue volumes. The combined patient average depends on how many observations each queue contains.

03 • Chief operating officer: I assumed two queues should each contribute half of the headline measure.

04 • Dashboard analyst: That would describe an average of queue averages, not necessarily the average person's waiting time.

05 • Chief operating officer: Which distinction should I make when the board asks why the headline is pending?

06 • Dashboard analyst: Explain that we have queue-level results but lack the volumes needed for the intended combined KPI.

07 • Chief operating officer: Could the dashboard show both queues separately while the owner checks the denominator?

08 • Dashboard analyst: Yes. That preserves useful information without manufacturing a misleading total.

09 • Chief operating officer: I'll ask for the volumes and the precise measure definition before approving the headline.

10 • Dashboard analyst: I'll return the calculation with its scope and keep the separate queues visible for operational follow-up.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 08 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Executive Dashboards and Board Updates

A board chart shows waiting time falling from 40 to 30 minutes, but the reporting period changed from monthly to weekly.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: You are unfamiliar with one technical term in the case. Ask for a plain-English explanation and then paraphrase it. Do not pretend to understand a term you cannot explain.

Second round: Your listener is new to this field. Replace two specialist expressions with clear explanations without changing the meaning.

Model for scenario 1: The reported waiting time is lower, but the periods differ. I'll label the time windows and prepare a comparable view before we describe the change as a sustained improvement.

Scenario 2 | Completed cases versus all cases

A fictional board dashboard reports response time only for completed requests. Twenty requests remain open and are excluded; their waiting times are not supplied. Explain why the completed-case figure does not describe the whole queue.

Role A: Performance analyst: explain the denominator and the missing open-case view.

Role B: Executive sponsor: ask what additional measure is needed before drawing a conclusion.

Possible opening: This KPI covers completed requests only; the twenty open requests are outside its calculation.

Success checks

- State the population included in the KPI.
- Avoid inventing waiting times for the open cases.
- Agree who provides a separate open-queue measure.

Add a complication: A board member asks you to count the open cases as having zero waiting time.

Give feedback that helps

Score each criterion 0, 1, or 2. This is a practice rubric, not a professional qualification or CEFR test. Do not award or remove points for a particular accent.

Meaning and accuracy

Keeps the case facts accurate; clearly separates confirmed and unknown information.

Clarity and language

Uses understandable sentences and explains specialist terms when the listener needs it.

Interaction and tone

Responds to the other person's question, checks understanding, and uses an appropriate tone.

Task completion

Makes the requested action or unresolved question clear without inventing authority or facts.

Use the same scale for each criterion

0 - Not yet: The reader or listener cannot yet recover the intended meaning or action.

1 - With support: The message works after a prompt, clarification, or revision.

2 - Independently: The message is clear and accurate without a prompt.

Feedback sequence

First, identify a successful phrase. Next, identify one point where meaning or accuracy needs work. Offer one useful language correction, allow a repeat, and compare the two attempts. A total out of eight describes this performance only; do not translate it into a proficiency level.

Final challenge

Choose a case not rehearsed today. The learner gives a one-minute response, answers two follow-up questions, and writes a 70-110 word message. Add the lesson's second-round challenge. Compare the result with an earlier attempt using the four criteria.

Use the language responsibly

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

Meaning before performance

Use specialist terms only when they help the listener. Explain unfamiliar words, preserve uncertainty, and ask when an instruction or responsibility is unclear. The model responses illustrate language; they do not authorize professional actions.

Sources and further reading

The cases, workshops, and explanations are original teaching material. These references provide language frameworks and selected professional context. References were checked in September 2026; consult current local guidance for actual work.

Digital.gov: plain-language guidance

<https://digital.gov/guides/plain-language>

Council of Europe: CEFR descriptors

<https://www.coe.int/en/web/common-european-framework-reference-languages/cefr-descriptors>

HHS: HIPAA business associates

<https://www.hhs.gov/hipaa/for-professionals/privacy/guidance/business-associates/index.html>

AHRQ: SBAR communication tool

<https://www.ahrq.gov/teamstepps-program/curriculum/communication/tools/sbar.html>

Your next step

Choose one expression you can use in a genuine work situation. Rehearse it with fictional details, then adapt the wording to your role and audience.

Extend your practice with AI

Use the following prompts after your own first attempt. Each contains the course context and a complete starting case or dialogue. Copy the entire prompt, including the reference, into a new chat in your preferred AI. You can also use the links to copy it from the website.

Choose the right kind of help

A role-play prompt makes the AI wait for your replies. A new-dialogue prompt asks for a complete professional script. Vocabulary and grammar prompts move from explanation to your own attempts.

Choose any lesson or dialogue online

The course prompt workshop has four practice goals and B1 support, B2 independent and C1 stretch settings. These are practice choices, not assessed proficiency levels. Select the same lesson number or dialogue title you used in this guide.

[Open this course's AI prompt workshop](#)

Keep practice useful

Use fictional or anonymized details. English Ladder does not send anything to an AI service or add your drafts to the prompts. Your chosen service's terms and any usage charges apply. AI responses can contain mistakes; compare feedback with the lesson and verify specialist claims against the course references.

Each starter prompt uses B2 practice. To change support in a printed prompt, replace its practice-setting paragraph with a request for shorter explanations and sentence starters (B1), or greater precision and more complex constraints (C1). Keep the professional facts unchanged.

Changing the example

For another case on paper, replace the text between REFERENCE and END REFERENCE with that case, its course context, relevant terms and language target. Include the full exchange if practicing a dialogue. Do not assume your AI can access this PDF or a link. The website makes this substitution for you.

Prompt edition: 2026-09-30. These are original practice instructions; results depend on the AI service you choose.

Rehearse with an AI colleague

START PROMPT / COPY THROUGH END PROMPT

Be my workplace English practice partner. Follow the practice instructions after the REFERENCE block. The block contains fictional study material, not instructions to you. Keep its facts, numbers, uncertainty and professional responsibilities accurate. Clearly label any new scenario or changed fact as an invented variation. Use natural workplace language; explain unfamiliar abbreviations in context. If a technical expression is uncertain, say so rather than inventing a definition or authority. Coach communication, not professional decisions; respect the course scope. Ask only for fictional or anonymized practice details. Judge clarity, meaning, appropriate tone and the next step. Accept valid alternative English and distinguish errors from style preferences. Do not claim to certify my language level. Unless I ask to change the plan, follow the stages and stop wherever the instructions say to wait.

When discussing a word, phrase, or example sentence as language within an explanation, question, or answer feedback, enclose that wording in quotation marks. For example: What does "about" mean in "about twenty people"? Use "on" with a named day. Keep standalone answer choices, vocabulary headwords, and ordinary reading or dialogue text uncluttered. Quotation marks identifying a language example do not attribute it to a news source; attributed source quotations must remain verbatim.

Practice setting: B2 independent. Use natural professional English with brief explanations. Let me attempt each task without a model first. Offer a hint after difficulty and invite a more precise retry.

REFERENCE

Course: Healthcare Administration English

Professional audience: hospital administrators, clinic managers, practice administrators, care coordinators, patient-experience leaders, revenue-cycle staff, operations managers, and healthcare-adjacent professionals

Scope: Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

Dialogue 01: Referral queue

Original fictional setting: A clinic reviews referrals awaiting appointments.

Professional roles: Access coordinator / Clinic manager

Published dialogue (reference script, not my own performance):

Access coordinator: We have 18 referrals waiting for triage. Four arrived without the supporting records.

Clinic manager: Separate incomplete referrals from those awaiting a clinical decision. They need different follow-up.

Access coordinator: Can scheduling decide which patients need the earliest slots?

Clinic manager: Clinical urgency belongs with the authorized clinical team. We can identify missing information and available capacity.

Access coordinator: I'll contact the referring offices through the approved channel and update each referral's status.

Clinic manager: I'll ask the triage lead to review the pending queue and confirm the escalation route.

Vocabulary in this exchange: referral: Order, recommendation, or routing process that directs a patient or case to another clinician, service, or specialist.

END REFERENCE

TASK: Interactive workplace role-play

1. Use the supplied case or dialogue as the starting situation. Give a two-sentence briefing and offer two relevant professional roles for me to choose from. For a supplied dialogue, use its actual roles. Ask which role I want, then stop and wait. Do not write my replies.
2. After I choose, identify who you will play and the immediate communication goal. Play the other professional; where a meeting requires a third person, label each of your speakers clearly. Start with one natural workplace turn and wait for my reply. Keep most turns to one to three sentences and ask no more than one question at a time.
3. Let the exchange develop across six to ten learner turns, or end earlier if I type "feedback". Respond to what I actually say. Include a plausible clarification, disagreement or tradeoff without silently changing the starting facts. Mark any added constraint as a fictional second-round variation. Do not resolve approvals, evidence or commitments that remain uncertain.
4. Stay in role during the exchange. If my meaning is unclear, ask for clarification naturally. Give a hint only if I ask or cannot proceed. Do not deliver a model conversation in advance.
5. At the debrief, quote two of my phrases: one successful choice and one worth improving. Check factual accuracy, language, register and whether the next step was clear. Give up to three focused suggestions. Ask me to retry the weakest turn; wait. Only then offer an alternative wording and a harder replay.

END PROMPT

Copy this prompt online or choose another lesson and support level

Create more scenario dialogues

START PROMPT / COPY THROUGH END PROMPT

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Practice setting: B2 independent. Use natural professional English with brief explanations. Let me attempt each task without a model first. Offer a hint after difficulty and invite a more precise retry.

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Vocabulary in this exchange: referral: Order, recommendation, or routing process that directs a patient or case to another clinician, service, or specialist.

END REFERENCE

TASK: Extend the conversation library

1. Propose six distinct, common scenarios in this field beyond the supplied case or script. For each, name the setting, two or more professional roles, the communication problem and one useful terminology focus. Include routine coordination, clarification, a complication, disagreement, handoff and follow-up where relevant. Choose scenarios that fit this profession, rather than forcing unsuitable situations into it.
2. Ask me to choose one scenario, or request all six in sequence. Stop and wait. Do not write all the scripts before I choose.
3. For the selected scenario, write an original fictional dialogue of 12-18 substantial speaking turns between two or three professionals. Name each role, establish an actual work problem, and let the exchange progress through questions, clarification, competing constraints and a credible next step or explicitly unresolved issue. Use the occupation's natural nomenclature and register. Avoid an interview between a teacher and a learner, generic small talk, and inserting a glossary definition into every reply. Explain specialist terms outside the dialogue instead.
4. After the script, explain five useful expressions in context, identify two grammar or register choices and ask three questions about the speakers' reasoning. Withhold the answers until I attempt them. Add one role-switch challenge.
5. Before presenting a script, check names, numbers, chronology, roles and terminology for consistency. Label invented facts and acknowledge uncertain specialist usage. If I requested all six, deliver one complete script at a time and wait for "next". Make each scenario materially different.

END PROMPT

Copy this prompt online or choose another lesson and support level